

1. Administrative & Study Identification

Full Study Title: Study of Ianalumab Versus Placebo in Addition to First-line Corticosteroids in Primary Immune Thrombocytopenia (ITP) **Short Title/Acronym:** VAYHIT1 **Protocol Number:** CVAY736I12301 **NCT Number:** NCT05653349 **Posting ID:** 13175185292206996 **Sponsor/Company:** Novartis **Investigational Product:** Ianalumab (VAY736) **Phase of Development:** Phase III **Trial Status:** Active, Not Recruiting (ACTIVE_NOT_RECRUITING) **Medical Monitor:** Novartis Clinical Operations

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the effect of two different doses of ianalumab versus placebo in addition to first-line corticosteroids in maintaining a platelet count ≥ 30 G/L and extending the time to treatment failure in adult participants with primary ITP. **Secondary Objectives:** To evaluate the Complete Response (CR) rate, overall Response (R) rate, time to complete response, duration of response, and stable response at 6 months in each treatment group.

2.2 Background & Rationale Primary Immune Thrombocytopenia (ITP) is characterized by a low platelet count due to immune-mediated destruction of platelets, increasing the risk of severe bleeding events. Current first-line medical therapies (like corticosteroids) rarely lead to durable responses after discontinuation. Ianalumab is a novel investigational, fully human monoclonal antibody targeting the B-cell activating factor receptor (BAFF-R), driving B-cell depletion and functional inhibition. This study addresses the medical necessity of providing a durable, long-term disease control mechanism to reduce the burden of continuous treatments and unpredictable relapses in ITP patients.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-blind (Quadruple blind: Participant, Care Provider, Investigator, Outcomes Assessor) **Randomization:** Randomized **Trial Status:** ACTIVE_NOT_RECRUITING (Active but not currently recruiting)

3.2 Planned Enrollment & Duration Target N: 226 participants **Number of Sites:** Multi-center (Global) **Estimated Study Start:** 03-Feb-2023 **Estimated Primary Completion:** 24-Apr-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Signed informed consent prior to participation in the study. 2. Male or female participants aged ≥ 18 years on the day of signing informed consent. 3. Confirmed diagnosis of Primary ITP within 3 months before initiating first-line ITP therapy. 4. Platelet count < 30 G/L before starting any first-line ITP therapy (corticosteroids, IVIG). 5. Evidence of response (platelet count ≥ 50 G/L) to corticosteroids (+/- IVIG) at any time prior to randomization. Note: Platelet count measured within 7 days of platelet transfusion will not be considered as a response.

4.2 Exclusion Criteria 1. Evans syndrome or any other cytopenia (patients with low grade anemia related to bleeding or iron deficiency are eligible). 2. Current life-threatening bleeding. 3. Previous ITP treatment (including splenectomy, prior B-cell depleting therapy like rituximab), except for corticosteroids and/or IVIG initiated as first-line therapy for up to 28 days before randomization and rescue corticosteroids and/or IVIG given prior to confirmed diagnosis of primary ITP. 4. Absolute neutrophil count < 1.0 G/L at randomization. 5. Participants with concurrent coagulation disorders and/or receiving anti-platelet or anticoagulant medication with an exemption of low dose of acetylsalicylic acid.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Ianalumab lower dose (3 mg/kg) or higher dose (9 mg/kg) versus placebo. Administered once a week up to Week 9, and every other week thereafter for a total of 16 weeks (total 4 doses of ianalumab/placebo). **Route of Administration:** Intravenous (Ianalumab or Placebo). **Duration of Treatment:** 16-week treatment period, followed by a follow-up period of up to 2 years (or up to 39 months).

5.2 Concomitant & Prohibited Therapy Permitted: First-line standard-of-care corticosteroids (oral or parenteral). **Prohibited:** Other B-cell depleting therapies, non-permitted anticoagulants, or initiation of second-line ITP therapies unless determined as a rescue treatment.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Platelet count verification, response to corticosteroids evaluation
Baseline	Day 0	Randomization, First IV Dose (Ianalumab/Placebo), Corticosteroid Continuation	

| **Interim** | Weeks 1-16 | Safety/Efficacy Labs, IV infusions (Weekly up to Week 9, then Q2W) | | **End of Study / Follow-up** | Up to 39 Months | Efficacy and safety monitoring every 4 weeks up to 2 years, then every 12 weeks until treatment failure or study end |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Time from randomization to treatment failure (TTF). Treatment failure is defined as platelet count ≤ 30 G/L later than 8 weeks from randomization, need for rescue treatment later than 8 weeks, start of second-line ITP therapy, or death. **Measurement Method:** Central/Local Laboratory platelet counts and continuous clinical evaluation up to 39 months.

7.2 Secondary & Exploratory Endpoints * Complete Response (CR) rate in each treatment group. * Overall Response (R) rate in each treatment group. * Time to complete response in each treatment group. * Duration of response and need of rescue treatment.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous AE evaluation throughout the 16-week double-blind treatment period and the prolonged safety follow-up phase. **Serious Adverse Events (SAEs):** Routine 24-hour reporting standards for any life-threatening events or severe bleeding episodes. **Data Monitoring Committee (DMC):** An independent Data Monitoring Committee is utilized to monitor patient safety and treatment efficacy.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) for evaluating the primary endpoint (TTF); Safety Set for patients receiving at least one dose of the investigational product. **Sample Size Justification:** $N=226$ participants; adequately powered ($\alpha = 0.05$, expected Power = 80-90%) to detect statistically significant improvements in time to treatment failure against the placebo arm. **Handling of Missing Data:** Standard imputation applied; patients lost to follow-up are censored at the date of last contact for the TTF primary endpoint.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Standard clinical site monitoring to assure

blinding integrity and drug accountability. **Audit Trail:** Robust electronic Case Report Form (eCRF) infrastructure with complete audit tracking for safety data, treatment failures, and rescue medication triggers.

11. Study Identifiers & Governance

Primary ID: CVAY736I12301 **Posting ID:** 13175185292206996 **Registry Number:** NCT05653349 **Sponsor/Lead Organization:** Novartis **Study Title:** Study of Ianalumab Versus Placebo in Addition to First-line Corticosteroids in Primary Immune Thrombocytopenia (ITP) **Official Regulatory Title:** A phase III, randomized, double-blind study of ianalumab (VAY736) versus placebo in addition to first-line corticosteroids in primary immune thrombocytopenia (VAYHIT1)

12. Clinical Framework (ITP Specific)

Primary Condition: Primary Immune Thrombocytopenia (ITP) **Preferred Name (UMLS):** Immune Thrombocytopenia, Primary / Autoimmune thrombocytopenia **Semantic Type:** Disease or Syndrome **Clinical Codes:** MeSH: D011681 | HPO: HP:0001973 | SNOMED ID: 128091003 **Diagnosis Threshold:** Platelet count ≤ 30 G/L **Medical Methodology / Drug Name:** First-line Corticosteroids (Standard of Care) / Combinations of corticosteroids (ATC Codes: D07XB30, D07AB30) **Optimization Target:** Maintaining platelet count ≥ 30 G/L and prolonging time to treatment failure

13. Study Procedures & Methodology

Management Pathway: Targeted B-cell depletion via BAFF-R blockade concurrently with established corticosteroid regimens. **Education Protocol (HCP):** Investigator training on quad-blinding mechanisms, IV protocols, and rescue treatment parameters. **Education Protocol (Patient):** Education on reporting bleeding episodes, recognizing relapse symptoms, and compliance with the intensive site-visit schedule. **Quality Audit Mechanism:** Continuous safety reviews by the DMC, specifically tracking the initiation of rescue treatments and platelet count variations.

14. Observation & Follow-Up Schedule

Total Duration: Up to 39 months post-randomization **Onsite Visit Cadence:** Weekly until Week 9; Every 2 weeks (Q2W) until Week 16. **Follow-Up Monitoring Frequency:** Every 4 weeks (Q4W) for the first 2 years, then every 12 weeks (Q12W) until study completion. **Checklist Review Interval:** Routine platelet level monitoring and concomitant medication check at every visit to detect protocol-defined treatment failure.

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---	
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]			Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]					